جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : نادية عزت سمعان رزق
المحافظة : القاهره **الحي / المدينة :** المعصره **العنوان :** عزبة خليل
رقم البطاقة : 26808130100509 **رقم التليفون :** 01203784224 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 13-08-1968 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: نادية عزت سمعان رزق
National ID: 26808130100509

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Patient Name	: الأستاذه / ناديه عزت سمعان رزق	Request No.	: 13001108
Patient No.	: 26808130100509	Request Date	: 17/07/2023
Path No.	: 2287/23	Verification Date	: 31/07/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 54 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Ali mostafa

Pathology Report

Clinical Data :
Right breast cancer post NACT.

Nature of the specimen :
Right MRM specimen collected at 17/7/23.

Gross Examination:
Right MRM specimen measured 18x17x4 cm , with covering skin elipse measured 17x10 cm
- Tumor extent : An ill defined soft to rubbery greyish white tumour bed site
- Size: 7x4x1.5 cm
- Skin involvement: Nipple & areola were unremarkable grossly
- Distance from margins:0.7 cm away from deep margin
- Lymph nodes number and size:12 nodes up to 1x1 cm .

Microscopic examination:
- Histological type: Total tumor regression in a previously diagnosed carcinoma
- Angioinvasion: Not detected
- Lymph node involvement: Free lymph nodes (0/12) with sarcoidal granuolomatous reaction in one node
- Skin involvement : Free nipple and areola
- Resection Margins: Free deep margin
- Histologic response to therapy: Pathologic complete response

Diagnosis:
Right Breast cancer post NACT , MRM specimen :
-Pathologic complete response in a previously diagnosed invasive carcinoma
-Free nipple and areola
-Free deep margin

Verified by :
dr.esraa magdy

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema- Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name: نادیه عزت سمعان	
MR. : 2051	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 58-year-old individual who is post-menopausal and has a history of hypertension. The clinical history notes a prior diagnosis of a postmenopausal right-breast invasive ductal carcinoma, clinical stage T3N1, described as triple-positive. The patient received neoadjuvant chemotherapy with TCH-PX6, followed by a modified radical mastectomy in July 2023 with pathological complete response, then underwent postoperative radiotherapy and completed one year of HERCEPTIN/PERJECTA. Adjuvant therapy with ARIMIDEX and ZOMETA was administered for six months. In April 2026, computed tomography demonstrated marked ascites; pathology of the ascitic fluid showed an exudative inflammatory pattern. Tumor markers were elevated (CA125 250 U/mL, later 292 U/mL; CEA 2.8 ng/mL; AFP 4 ng/mL). Laboratory values showed serum creatinine decreasing from 2.3 mg/dL to 1.8 mg/dL. Imaging revealed diffuse omental thickening with ascites, multiple tiny pulmonary nodules on CT chest-abdomen-pelvis, and no suspicious brain lesions on MRI; MRI of the pelvis could not be performed due to the elevated serum creatinine.

Immunohistochemistry demonstrated estrogen receptor 8/8, progesterone receptor 8/8, and HER2 +2 amplified. The current imaging findings include diffuse omental thickening, ascites, and multiple tiny pulmonary nodules, with no brain metastases identified. The metastatic work-up is limited by the inability to obtain a pelvic MRI because of renal impairment.

The referring team recommended transvaginal ultrasound to exclude a primary gynecologic source, echocardiography with new laboratory studies including CA 15-3, a new bone scan and serum tumor markers (CA 15-3 and CA 125), and a PET-CT if approved. The MDT discussed initiating taxol-herceptin therapy in the interim. The MDT outcome and plan are to obtain a new contrast-enhanced CT of the chest, abdomen, and pelvis with biopsy of the omental thickening, repeat tumor markers CA 15-3 and CA 125, and to commence taxol-herceptin while awaiting further diagnostic results.

تحريرا في ,

التاريخ :	23-09-2026	**الوقت :**	

الطبيب المعالج :

يعتمد ,

Patient Name	: الأستاذه / ناديه عزت سمعان رزق	Request No.	: 13001108
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Age	: 54 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Ali mostafa

Pathology Report

-Free lymph nodes (0/12) with sarcoidal granuolomatous reaction in one node
-Stage : ypT0 , ypN0

Prof.Gamal Abdelatty
Dr.Mennan Hossam

Verified by :
dr.esraa magdy

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema- Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

جامعه القاهره
معهد الاورام
قسم الباثولوجى

Patient Name :	ناديه عزت سمعان رزق	**Pathology No :**	**13798/22**
Sex :	**Age :** 54	**Hospital No.**	**4414/13**
Clinical Data:	Tru-cut biopsy from right breast 12 o'clock malignant looking lesion.		
Department :	مريض عياده خارجيه وحده جراحه - ا	**Referring Dr. :**	

Electronically Authorized by: YARA YASSER MOHAMED

Gross :

Multiple tissue cores, ranging in size from 1 to 1.36 cm, long.

Histologic :

Section examined reveals cores of breast tissue, infiltrated by an invasive breast carcinoma formed of irregular solid groups of malignant ductal epithelial cells with focal ductal differentiation. Anaplasia and mitosis are moderate. Surrounding stroma shows desmoplasia with evidence of focal lympho-vascular permeation. ER shows strong positive reaction in 100% of the tumor(score 8/8), PR shows strong positive reaction in 95% of the tumor(score 8/8) and HER-2 shows positive reaction, score 2+(SISH)is ordered.

Diagnosis Breast,Right,Biopsy:Invasive duct carcinoma. ,Grade 2.

Category Malignant primary site (Single tumour)

REPORT WAS SIGNED ON 01/11/2022

Verbal Revision Hazem Abd Elazim Sobhi

Typist sana'a abdel aziz ismael

Pathologists :

Dr. Mahitab Ibrahim Eltohamy

Dr.

Dr. Hazem Abd Elazim Sobhi

Date of Issue : 07/12/2022

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

EZZAT SMAAN RZQ, NADYA, Patient ID: 113004414 DOB: 13-Aug-1968 Date: 23-Jan-2023

Patient Lastname: Ezzat Smaan Rzq
Patient Firstname: Nadya
HIS Patient ID: 113004414
RIS Patient ID: 93741
Date of Birth: 13/08/1968

Procedure: 70030, Bone Scan
Medical Question:
Clinical Comment:

========== REPORT TEXT ==========
99mTc MDP Bone Scan Report (23/01/2023)
* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP
Revealed:

" No definite evidence of active osseous deposits could be seen over the surveyed skeleton.
" Degenerative changes are noted over the dorso-lumbar vertebrae.
" Arthritic changes are noted over both shoulder and both knee joints.
" Both kidneys are visualized.
Conclusion:
" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obliged
Dr. Mayadah Ashraf
Omnia Talaat.MD

========== REPORT TEXT ENDE ==========

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name: Nadia Ezzat Saman

Date: 19/12/2022

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: right invasive duct carcinoma grade II

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: moderate nodular decrease the sensitivity of the study

ACR: C (heterogeneously dense)

LEFT BREAST

- **Low energy (Non contrast) images**: diffuse scattered amorphous microcalcifications without grouping, linear, segmental or regional distribution
- show no masses, asymmetry or distortion
- **Recombined (Post contrast) images**: show, no pathological uptake.
- Normal skin / nipple / areolar complex.

RIGHT BREAST:

- **Low energy (Non contrast) images** : upper central round dense lobulated mass lesion with coarse heterogeneous microcalcifications seen within the lesion.
- UOQ amorphous microcalcifications shows linear & grouping distribution seen anterior to the index lesion .
- **Recombined (Post contrast) images** upper central anterior one third heterogeneous enhancing mass lesion with anterior non mass enhancement , this corresponding to the index mass & adjacent microcalcifications distribution , both measures 6.3x5.8x7.3cm located at about 1.5cm from the nipple, **complementary** US shows ***intraductal*** extension of the lesion reaching the nipple
- Normal skin / nipple / areolar complex.
- **Complementary ultrasound:** pathologically enlarged level I axillary lymphnodes (9) in number the largest is globular in shape measures 2x1.8cm with hilum loss .

IMPRESSION:

LEFT BREAST: Diffuse scattered probably benign amorphous microcalcifications without grouping, linear, segmental or regional distribution for 6 months follow up (BIRADS3 = probably benign)

RIGHT BREAST: upper central mass & non mass enhancement as described with pathologically enlarged level I axillary lymph nodes (BIRADS6 = pathologically proved)

Best regards,

Dr. Marwa Ramzy , MSc

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

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أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nadia Ezzat Samaan Rezk	Date : 18/03/2023
Exam : ultrasound BREAST (BOTH SIDE)	Age : 54 Year(s)
Medical ID: 4552/2022	

FOLLOW UP BILATERAL BREAST ULTRASOUND

INDICATION:

- Follow up post chemotherapy for right breast neoplastic process.
- In correlation to the previous study of 19/11/2022

REPORT:

Regressive course as regards the size of the previously detected:

- Right breast 12 o'clock spiculated hypoechoic masses , currently measuring 30 x 19 mm compared to 45 x 48 mm in the previous study.
- Left pathological axillary LNs with diffuse cortical thickening ,the largest currently measures 19 mm compared to 38 x 17 mm in the previous study.

- No left breast suspicious mass lesions.
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- left non specific axillary lymph nodes.

OPINION:

- **Follow up post chemotherapy for a case of right breast malignancy, showing good therapeutic response, with regressive course as regards the loco-regional and nodal neoplastic burden since the last study, with residual as described (BIRADS6).**

Dr.Wafaa Ali

Signature Temp

18/03/2023 1:23 PM

Ultrasound
[[DOCTYPE:ULTRASOUND]]

EZZAT SMAAN RZQ, NADYA, Patient ID: 113004414 DOB: 13-Aug-1968 Date: 23-Jan-2023

Patient Lastname: Ezzat Smaan Rzq
Patient Firstname: Nadya
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Ministry Of Health
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أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nadia Ezzat Samaan Rezk	Date : 26/11/2022
Exam : C.T CHEST WITH CONTRAST	Age : 54 Year(s)
Medical ID:	

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN, AND PELVIS: -

Findings:

I Chest:-

- Right breast UIQ irregular mass lesion measures 4 x 4,.3 cm with pathological enlarged right axillary lymph nodes at level I, II and III largest measures 1.9 x 2.1 cm
- Few bilateral scattered 1-2 mm pulmonary nodules.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

II Abdomen & pelvis:

- Enlarged liver with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Right iliac bone tiny sclerotic focus likley bone island.
- Dorsolumbar spondylosis.

CT Scan
[[DOCTYPE:CT_SCAN]]

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أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nadia Ezzat Samaan Rezk	Date : 26/11/2022
Exam : C.T CHEST WITH CONTRAST	Age : 54 Year(s)
Medical ID:	

Opinion:

- **Right breast neoplastic lesion with axillary lymphadenopathy.**
- **Tiny pulmonary nodules likley non specifc.**
- **No pulmonary or hepatic metastatic deposits.**

Dr. M. Gomaa, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

EZZAT SMAAN RZQ, NADYA, Patient ID: 113004414 DOB: 13-Aug-1968 Date: 23-Jan-2023

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